جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : عواطف عبدالرحمن عفيفى العرضى
المحافظة : القاهره **الحي / المدينة :** المطريه **العنوان :** ئ
رقم البطاقة : 25710110103206 **رقم التليفون :** 01210665034 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 11-10-1957 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بعنق الرحم
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بعنق الرحم
خطة العلاج : أورام كيماوي علاج تدعيمي وتلطيفي [6656] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: عواطف عبدالرحمن عفيفى العرض
National ID: 25710110103206

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

A patient of cervical cancer for Supportive and palliative Treatment

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Visit Number:	61126517439	Client Name:	نقابة المحامين -القاهرة
Patient Name:	عواطف عبد الرحمن عفيفي	Referred by Prof:	-
Age / Sex:	58 year / Female	Exam Date:	12 Jul 2026
Patient ID:	2141641553	Printed:	14 Jul 2026 11:10:47 AM

CT

CT SCAN OF THE ABDOMEN AND PELVIS WITHOUT CONTRAST:

TECHNIQUE: Multislice axial CT images of the abdomen and pelvis were obtained followed by multiplanar reconstruction of the source images. No contrast was used.

CLINICAL HISTORY: A -57-year-old diabetic and hepatic with history of thyroidectomy. Now she complains of chronic constipation

FINDINGS:



- The liver is cirrhotic with irregular borders, however showing homogenous CT density and no focal lesions on a non-contrast basis study.
- No intra or extra hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, both kidneys, suprarenal glands, and IVC.
- Inadequately distended urinary bladder showing no stones, masses or diverticulae.
- **Bilateral mixed and solid adnexal lesions. The right lesion measures 4.2x3.8 cm and the left lesion measures 4.6x4.2 cm.**
- **Moderate to marked pelvi-abdominal ascites.**
- Few aorto-pulmonary window, subcarinal, mediastinal and abdominopelvic enlarged lymph nodes.
- Few pelvic phleboli
- Bilateral dorsal elastofibroma (incidental finding)

Scan Result

Name: عواطف عبدالرحمن عفيفى العرضى **MR. :** 12535	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 70-year-old individual with a history of thyroid cancer treated with thyroidectomy and radioactive iodine in 2006. Presenting with progressive abdominal pain and constipation, an MRI performed on 16/08/2026 revealed extensive bilateral complex solid-cystic adnexal masses with uterine involvement, highly suspicious for bilateral ovarian adnexal carcinoma. The masses are aligned with local pelvic extension and there is no definite preserved plane of cleavage from the anterior rectosigmoid junction, raising concern for bowel involvement. Multiple bilateral restricting hepatic lesions suspicious for hepatic metastases are noted, along with inter-regional peritoneal and retroperitoneal soft tissue changes, massive ascites, mild distal descending colon mural thickening, and indeterminate metastatic serosal involvement. Upper gastrointestinal evaluation was negative for malignancy. Colonoscopy was performed and was free of lesions. A CA-125 level of 99 was obtained for ascitic tap and cytology analysis. Biopsy of liver or adnexal mass was recommended.

On 10/09/2026, the imaging findings were reiterated, and cytology from ascites was reported positive for malignancy with immunohistochemistry compatible with a Müllerian origin. The plan was to initiate weekly Taxol and carboplatin on a private basis until further evaluation (MOHR).

A clinical pharmacy note on 10/09/2026 documented the patient's weight (95 kg), height (155 cm), and serum albumin (2 g/dL). The first week of therapy (w1) was prescribed Taxol 150 mg and carboplatin with an AUC of 2. The patient has completed 1 week of the Taxol/Carboplatin regimen to date.

تحريرا في ,

التاريخ :	**19-09-2026**	**الوقت :**	

الطبيب المعالج : نرمين مصطفي عبد القادر بهي الدين

يعتمد ,

معامل البرج

بالتأكيد..

Visit Number:	61126517439	Client Name:	نقابة المحامين -القاهرة
Patient Name:	عواطف عبد الرحمن عفيفي	Referred by Prof:	-
Age / Sex:	58 year / Female	Exam Date:	12 Jul 2026
Patient ID:	2141641553	Printed:	14 Jul 2026 11:10:47 AM

CT

- Scanned lung bases show moderate left pleural effusion, veiling and thick atelectatic bands.
- Atherosclerotic changes and calcifications of the aorta.

OPINION:

- **Cirrhotic hepatic changes**
- **Bilateral mixed solid and cystic adnexal lesions. For dedicated contrast study enhanced MRI examination and CA 125 correlation.**
- **Moderate to marked pelvi-abdominal ascites.**
- **Mild left pleural effusion.**

Dr .Nada Khaled

Professor. Sally Emad-Eldin, MD

Scan Result

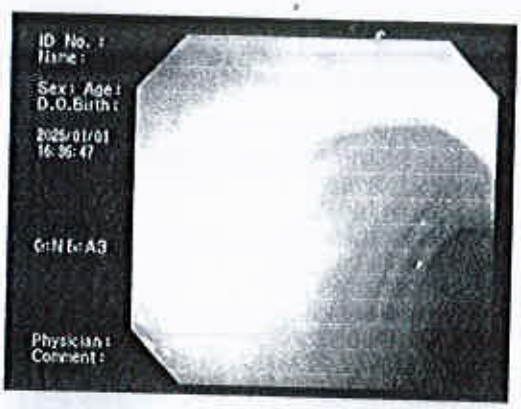
CT Scan
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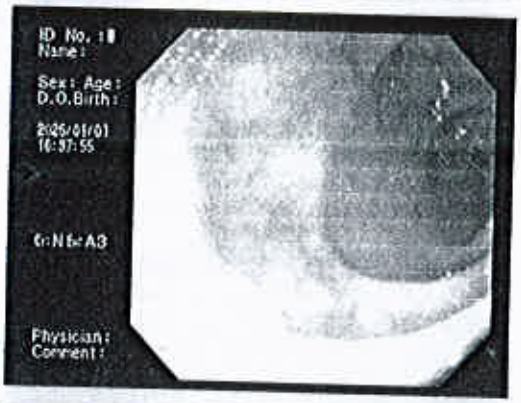
SCOPE
ENDOSCOPY
CENTER
مركز سكوب
لمناظير الجهاز الهضمي و الكبد

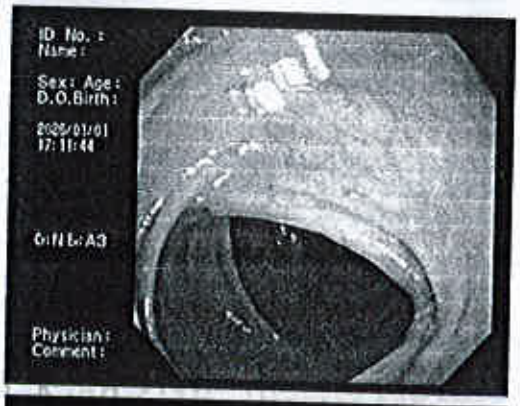
مركز سكوب
لمناظير الجهاز الهضمي و الكبد

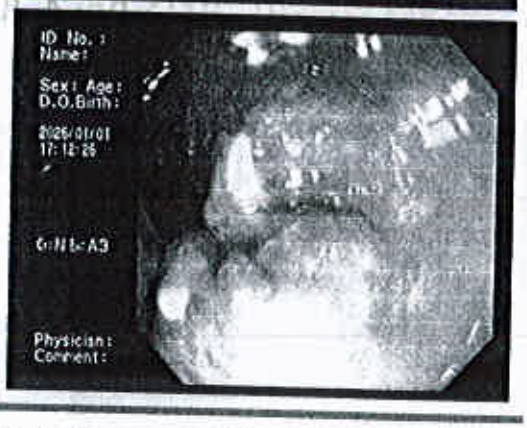
Colonoscopy Report

Patient Name:	عواطف عبدالرحمن عفيفي العرضى	**ID**	50736
Endoscopist:	Room 2	**DOB**	
2nd-Operator:		**Age:**	69
Date :	Thursday 01, January 2026	**Ref By**	









INDICATION FOR EXAMINATION

Screening for CRC

Diagnosis

PROCEDURE PERFORMED

Diagnostic

MEDICATIONS

Propofol

Preparations and Procedure Details

Colonoscopy is done up to hepatic flexure

FINDINGS:

Colon

The whole exposed mucosa looks normal. Very tough adhesions are causing colon fixation with difficult manipulation. Introduction to cecum was not possible.

Congested internal piles are seen.

Ileum

-

Conclusion

Normal examined par of colon.
Congested internal piles.

Recommendations:

Endoscopist: Prof. Tamer Elbaz

Mobile:01095552221 -01069999430 -01069997770
Landline: 37605830- 37605820
Website: www.scope-center.com
E-mail: Info@scope-center.com

١٢شارع مصدق - الدقى - برج امصدق الطبي

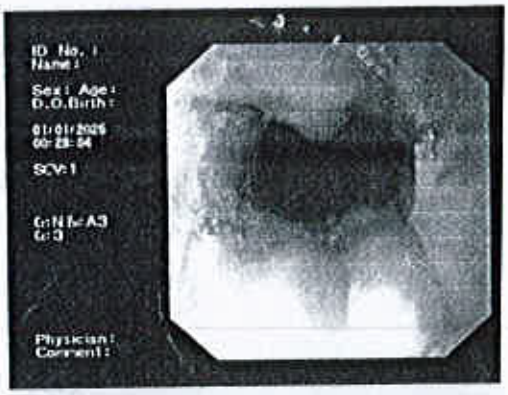
ENDOSCOPY
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SCOPE ENDOSCOPY CENTER
مركز سكوب لمناظير الجهاز الهضمي و الكبد

مركز سكوب
لمناظير الجهاز الهضمي و الكبد

Upper Endoscopy Report

Patient Name:	عواطف عبدالرحمن عفيفي العرضى	**ID**	50736
Endoscopist:	Prof. Tamer Elbaz	**DOB**	
2nd-Operator:		**Age:**	69
Date:	Thursday 01, January 2026	**Ref by:**	



INDICATION FOR EXAMINATION

PROCEDURE PERFORMED

Diagnostic

MEDICATIONS

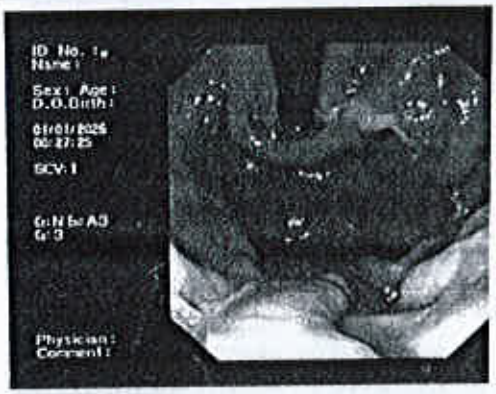
Propofol

Esophagus

Incompetent cardia. The lower end is hyperemic.

Stomach

The whole mucosa is hyperemic, more pronounced at antral region. Antral biopsies are taken for HPE.

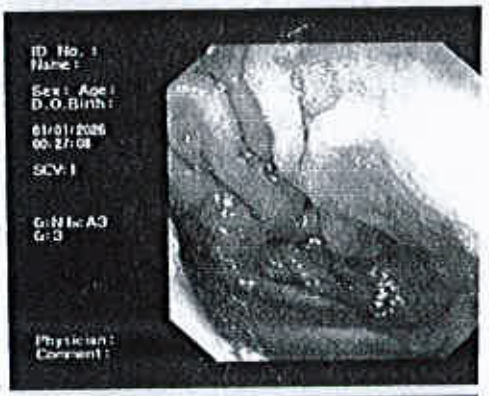


Pylorus

Rounded and active

Duodenum

The mucosa is hyperemic down to the third part



Conclusions

Mild reflux oesophagitis.
Pangastritis.
Duodenitis.

Recommendations



Endoscopist Prof. Tamer Elbaz

Mobile:01095552221 -01069999430 -01069997770
Landline: 37605830- 37605820
Website: www.scope-center.com
E-mail: Info@scope-center.com

اشارع مصدق - الدقى - برج مصدق الطبي

ENDOSCOPY
[[DOCTYPE:OTHER_ENDOSCOPY]]

[[OH:DOCTYPE:IHC]]
Immunohistochemistry

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : عواطف عبد الرحمن عفيفى العرضى **Analysis ID :** 1090053

Age : 68 Y : 1 M : 1 D **Gender :** Female **Conf. Code :** 4543

Referred By Prof. Dr. : هشام الغزالى

Clinical Diagnosis : Ascitis.

Nature of Specimen : Block (Ascitic fluid, Cytology, labelled as 1089356).

Receiving Date : 2026-08-27 **Patient ID :** 794520

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin block, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain .
The sections were treated against:
- Ck 7 \ PAX8 \ WT1 \ P53 \ ER (Estrogen receptor).

Results :

Tumor cells are POSITIVE for Ck 7.
Tumor cells are POSITIVE for PAX8.
Tumor cells are NEGATIVE for WT1.
P53 showed weak expression (mutation could not be assessed).
Sections treated for Estrogen receptors revealed NEGATIVE nuclear staining.

Conclusion :

Block, MALIGNANT ASCITES ARE COMPATIBLE WITH MULLARIAN ORIGIN.
-TUMOUR CELLS ARE NEGATIVE FOR ESTROGEN RECEPTORS (SCORE 0 / 8).
N.B: P53 WEAK EXPRESSION PREVENT PROPER ASSESSMENT OF P53 MUTATION, TISSUE BIOPSY IS MANDATORY FOR PROPER SUBCLASSIFICATION OF THE TYPE OF CARCINOMA.

Prof. Dr. Shady Elia Anis ***Prof. Dr. Elia Anis Ishak***

This report is electronically signed , any abrasion or modification would render it void .

DR ELIA ANIS LABS
عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٠١٢١٢٩٤٥١١٥ - ٠١٢٨٧٠٠٥٢١٧ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥

[[DOCTYPE:PATHOLOGY_REPORT]]
Pathology Report

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : عواطف عبد الرحمن عفيفى العرضى **Analysis ID :** 1089356
Age : 68 Y : 1 M : 1 D **Gender :** Female **Conf. Code :** 1038
Referred By Prof. Dr. : هشام الغزالى
Clinical Diagnosis : Ascitis
Nature of Specimen : Ascitic fluid, Cytology
Receiving Date : 2026-08-25 **Patient ID :** 794520

PATHOLOGY REPORT

Gross :

Two bags were received:
1- 2000 cc of semiturbid yellow fluid.
2- 1800 cc of semiturbid yellow fluid.

Microscopic :

Cytological examination of the films and cell blocks prepared from the centrifugate of the fluid received revealed red blood cells, inflammatory cells, scattered groups of epithelial cells with moderate nuclear pleomorphism arranged in tridimentional and acinar patterns in proteinaceous background.

Diagnosis :

Ascitic fluid, Aspirate, Cytology, POSITIVE FOR MALIGNANT CELLS.
CASE FOR IMMUNOPHENOTYPING.

Prof. Dr. Shady Elia Anis ***Prof. Dr. Elia Anis Ishak***

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DR ELIA ANIS LABS
عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٠١٢١٢٩٤٥١١٥ - ٠١٢٨٧٠٠٥٢١٧ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥

[[DOCTYPE:CT_SCAN]]
CT Scan

جوستاف روسي انترناشونال

Patient's Name: Awtif Abdulrahman Afifi Al-Ardi
Date: 2026-08-26
Referred by: Prof.Dr. gustave doctor
Hospital: Goustave Roussey International Egypt

MULTISCLICE C.T. EXAMINATION OF THE ABDOMEN AND PELVIS FOLLOWING ORAL CONTRAST ADMINISTRATION REVEALED :

- Average sized liver showing heterogenous parenchymal attenuation. Few bilobar hypodense focal lesions largest measuring 15 x 9 mm at segment IV. No dilated IHBRs.
- Multiple omental cakes are seen largest measuring 18 x 14 mm.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness. No stones or back pressure changes.
- Small bowel loops show multiple air/fluid levels with no significant distension.
- Bilateral bulky adrenal glands.
- Normal non-contrast C.T appearance of the spleen, pancreas.
- Mildly thickening of the pyloric wall reaching 25 mm in thickness.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Two bilateral adnexal cysts measuring 50 mm on the right side and 26 mm on the left side. The right side cyst is hyperdense.
- Normal appearance of the rest of pelvic organs.
- Multiple subcentimetric bilateral internal and external iliac lymph nodes.
- Moderate ascites.
- Basal chest cuts showed mild left pleural effusion with underlying subsegmental lung collapse and few tiny right lower lobe pulmonary nodules.

Opinion:

- **Chronic parenchymatous liver disease with few bilobar hypodense hepatic focal lesions for dynamic MR study.**
- **Multiple omental cakes.**
- **Mildly thickened pyloric wall for endoscopy.**
- **Multiple small bowel loops air-fluid levels yet with no evident distension for clinical correlation.**
- **Bilateral bulky adrenal glands.**

[[DOCTYPE:CT_SCAN]]
CT Scan

Patient's Name:	**Awtif Abdulrahman Afifi Al-Ardi**
Date:	**2026-08-26**
Referred by:	Prof.Dr. **gustave doctor**
Hospital:	**Goustave Roussey International Egypt**

- **Two bilateral adnexal cysts with the right side cyst being hyperdense ? hemorrhagic for MR assessment.**
- **Moderate ascites.**
- **Mild left pleural effusion with underlying subsegmental lung collapse and few tiny right lower lobe pulmonary nodules for dedicated study.**

Dr.sherif Wagdy

Dr.sherif Wagdy

[[DOCTYPE:MDT_FORM]]
MDT Form

GUSTAVE ROUSSY INTERNATIONAL EGYPT

Date Of Submission: ٩-١٢

Date Of MDT: 09/13/2026 15:41:42

General GRI-E MDT form

Patient Name :	Awatif عبدالرحمن عفيفى العرضى	**Age :**	69 Y
Sex :	○ Male ○ Female	**Comorbidity :**	DM HTN on L thyroxine due to thyroidectomy
ID No. :	25710110103206	**Telephone :**	01080917426
Family History :	+ Lymphoma	**Marital Status :**	◉ Married ○ Divorced ○ Single
Menopausal Status :	post	**Reason of MDT :**	
		MDT name::	

Medical Report

Diagnosis : History of thyroid cancer in 2008 and now metastatic ovarian cancer

Notes :

History: EXTENSIVE BILATERAL COMPLEX SOLID CYCSTIC ADENXAL MASSES WITH UTERINE INVOLVEMENT HIGHLY SUSPICIOUS FOR BILATERAL OVARIAN ADENXAL ,ALIGNANCY WITH LOCAL PELVIC EXTENSION NO DEFINITE PRESERVED PLANE OF CLEAVAGE FROM ANTERIOR RECTOSIGMOID JUNCTION RAISING CONCERN OF BOWEL INVOLVEMENT 'MULTIPLE BILATERAL RESTRICTING HEPATIC LESIONS SUSPICIOUS FOR HEPATIC METS ITTERGULAT PERITONEAL RETROPERITONEAL SOFT TISSUE CHANGES MASSIVE ASCITIS MILD DISTAL DESCENDING COLON MURAL THICKENING INDETERMINATE METATSTIC SEROSAL INVOLVEMENT SHOULD BE CONSIDERED
CYTOLOGY FROM aSCITIS : POSTIVE FOR MALIGNANCY
IHC :Malignant ascitis are compatible with mulerian origin
to start Taxol carboplatin weekly on private basis till MOHR

Investigations:

Recommendations: Taxol Carboplatin weekly for 12 weeks

MDT DECISION: Taxol Carboplatin weekly for 12 weeks

Signature	Hematology	Pediatric	Bone marrow transplant
د خك عبد العزيز	د محمد فتراوي		